Foundations NRSG 231 · Extra Review (OPTIONAL)

Read this first: Your Foundations professor uses **no textbook content**. Everything here comes from the Taylor textbook, so it is **optional extra review only, if you have time**. What's tested is **Guide v3 + her slides**. Study those first.

What's in here: only the parts of the old book-based guide (v1) that v3 does **not** already cover: Taylor reading details, Taylor tables/figures, and book-based NCLEX-style traps. Every topic that repeats v3 (her slides, the core tables, the lab-skill steps, the slide pictures) was left out.

Midterm: Wed Oct 21, 2026, 8:30 AM. "⚠ Conflict" = her slides and Taylor differ: go with her slides.

Ch. 25 Asepsis & Infection Control

From the assigned reading (Taylor)

- **Colonization vs infection**: normal flora are nonpathogenic residents; **opportunists** (e.g., E. coli from gut → UTI; C. diff) cause disease when defenses fail. C. diff present with no clinical signs = **colonization**; with manifestations = **infection** [Taylor p. 667].
- **Carriers**: asymptomatic reservoirs who can still transmit (e.g., HIV-positive person with no symptoms; maternal-fetal transmission) [Taylor p. 667].
- **Bacteria categories**: shape (cocci, bacilli, spirochetes); **gram-positive** (thick wall, stain violet) vs **gram-negative** (do not stain) — drives antibiotic choice; **aerobic** (need O_2) vs **anaerobic** [Taylor p. 664].
- **Viruses**: smallest, need electron microscope; **antibiotics have no effect**; antivirals (e.g., influenza, HSV-1 in the **prodromal** stage) shorten illness; HIV treated with ART [Taylor p. 664].
- **Fungi/parasites**: athlete's foot, ringworm, yeast (antifungals; many resistant); malaria via infected mosquito [Taylor p. 665].
- **Endemic vs pandemic**: endemic = predictable in one region/population; pandemic = global outbreak of a novel virus (COVID-19) [Taylor p. 666].
- **Table 25-1 examples**: Lyme disease (ticks, indirect/vector contact); E. coli (feces/food/water, direct/indirect); HBV (blood/body fluids, direct); HIV (blood, semen, vaginal secretions, breast milk — direct contact); TB (sputum — **airborne**); Salmonella (intestinal tracts — direct/indirect); *S. aureus* (skin, mouth, nose, throat — direct/indirect; abscesses, endocarditis, osteomyelitis, bacteremia) [Taylor Table 25-1, p. 669].
- **Defense mechanisms**: first line = skin, mucous membranes, normal flora; then inflammatory and immune responses. Hallmarks of acute inflammation = **redness, heat, swelling, pain, loss of function**; vascular then cellular phase; neutrophils phagocytize; exudate = serous, sanguineous, or purulent; repair by regeneration or scar tissue [Taylor p. 670].
- **Immune response**: **antigen–antibody (humoral) immunity** plus **cell-mediated immunity** (lymphocytes) [Taylor p. 670] [Taylor p. 671].

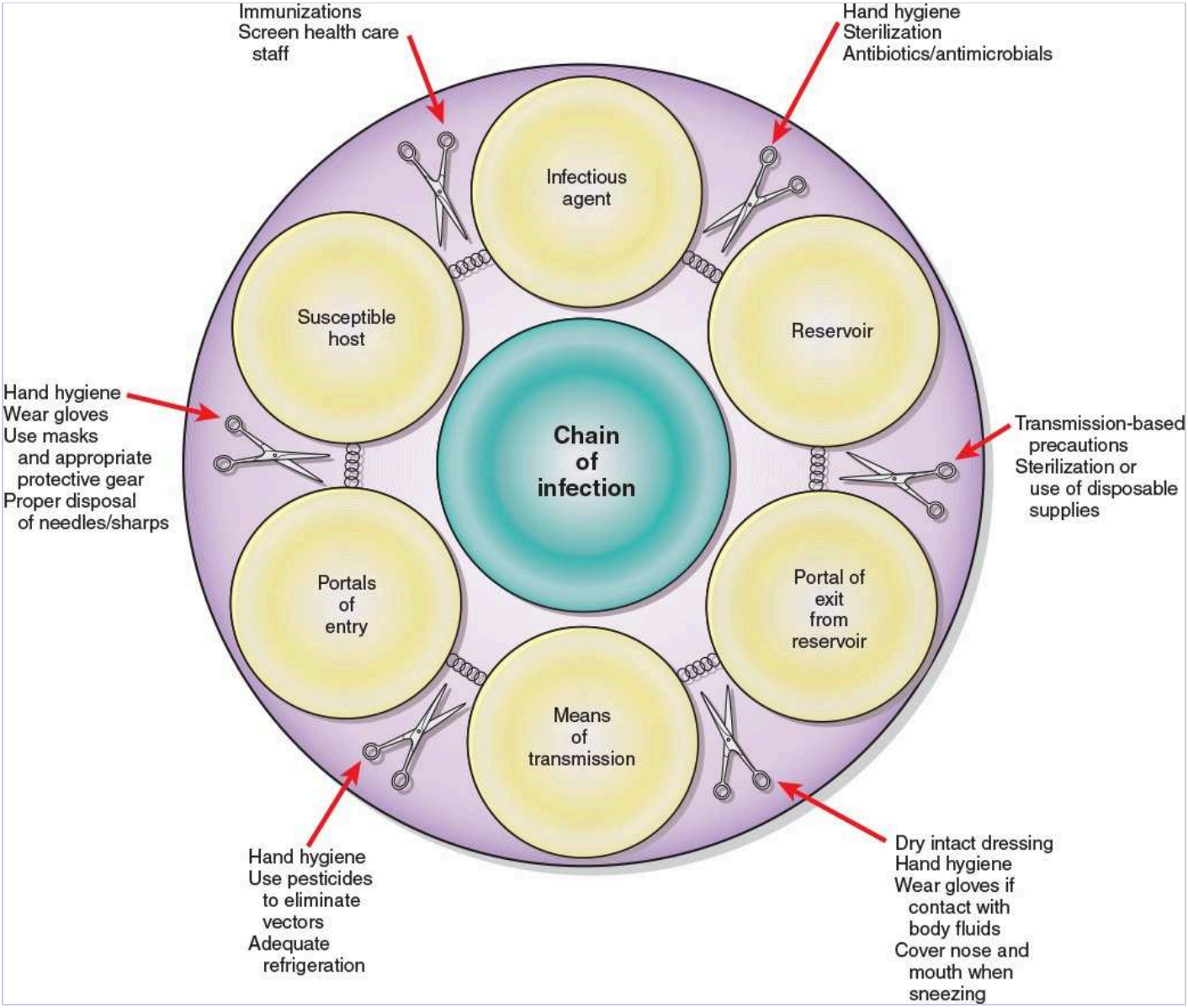
- **Localized vs systemic infection signs**: localized = redness, swelling, warmth, pain/tenderness, loss of function; systemic = fever with increased pulse & respirations, lethargy, lymph node tenderness/enlargement, anorexia [Taylor p. 672].
- **Older adults**: atypical infection presentation — confusion, disorientation, lethargy, anorexia, delayed fever response, falls, incontinence, failure to thrive; age risks include pulmonary, UTI, and skin infections (decreased cough reflex, incomplete bladder emptying, thinning epidermis) [Taylor Focus on the Older Adult, pp. 671–672].
- **Box 25-2 medical asepsis basics**: carry soiled linens away from clothing; nothing on the floor; tissue for coughing/sneezing; move equipment away when dusting/brushing; avoid raising dust (damp cloth; don't shake linens); **clean least soiled → most soiled**; wrap moist items in waterproof bags; pour liquids directly into drain; sterilize contaminated equipment; short clean nails, no rings with grooves, hair restrained; follow standard/transmission-based precautions [Box 25-2, p. 674].
- **Hand flora**: **transient** bacteria (loose, in grease/dirt, greatest under nails — easily removed by handwashing) vs **resident** bacteria (skin creases, cling tenaciously, need friction/brush, less susceptible to antiseptics) [Taylor Table 25-2, p. 676].
- **Nails**: no artificial nails for patient care (OR violation = citable); natural nails < 1/4 inch [Taylor p. 676].
- **Hand hygiene method & timing**: alcohol-based handrub (60%–95% alcohol) when hands are **not visibly soiled** — apply recommended amount (≈3 mL), cover all surfaces/interdigital areas/nails, rub **until completely dry (at least 20 seconds)**; wash with **antimicrobial soap and water for at least 20 seconds** when hands are visibly soiled or contaminated with blood/body fluids, and before eating and after the restroom; longer scrub if visibly soiled [Taylor p. 677, Guidelines for Nursing Care 25-1].
- **Alcohol rubs are ineffective against** *Cryptosporidium*, **norovirus, and C. diff (spores)** — use soap and water; contact precautions and gown/gloves until diarrhea stops [Taylor p. 677].
- **Gloves**: not a substitute for hand hygiene; single use; change between tasks and before moving contaminated → clean; warmth/moisture inside gloves promotes bacterial growth; hand hygiene before and after glove use; **double gloving** reduces perforation (71% in OR); do not touch charts, keyboards, phones, or leave the unit in gloves [Taylor p. 682].
- **Latex** (Box 25-3): irritant contact dermatitis (not an allergy), allergic contact dermatitis (type IV, 24–96 hr), true latex allergy (type I, anaphylaxis); ~10% of health care workers sensitized; powder binds latex protein and becomes airborne → move to powder-free gloves; medical alert bracelet; no cure, avoidance only [Box 25-3, pp. 682–684].
- **Masks/eyewear**: surgical mask filters expired air (large droplets >5 µm, droplets travel <3 ft, up to 6 ft); particulate respirator (N95) filters inspired air and particles ≤5 µm; **fit test required** and **seal check each use**; AII room for TB = negative pressure, 6–12 air changes/hour, HEPA or exhaust to outside; change mask before it becomes damp; goggles/face shield with side shields for splash risk (plain glasses not acceptable) [Taylor pp. 684–685].
- **HAI terms**: **exogenous** (organism from other people), **endogenous** (from the patient's own flora), **iatrogenic** (result of a treatment/diagnostic procedure); CMS non-reimbursement for preventable HACs; mandatory public HAI reporting in most states [Taylor p. 678] [Taylor p. 679].
- **Bundles** = 3–5 evidence-based practices implemented together; CAUTI risk rises 3%–7% per day the catheter stays in; VAP bundle includes HOB 30°–45°, regular oral care, changing ventilator circuit only as needed [Taylor pp. 678–679].
- **MDROs**: MRSA (HA-MRSA: IV vancomycin first line; CA-MRSA: often incision & drainage, culture before antibiotics); VRE (feces/urine/blood; linezolid or daptomycin); CRE (40%–50% mortality;

contact precautions); CRAB (clean/disinfect environment); C. diff (watery diarrhea, fever, cramping; sporicidal terminal cleaning, soap and water, antibiotic stewardship) [Taylor pp. 679–681].

- **Disinfection vs sterilization**: disinfection destroys pathogens **except spores**; sterilization destroys **all** microorganisms **including spores**. Sterilize items entering a sterile body part. Methods (Table 25-3): steam/autlave, boiling ≥10 min, dry heat 350°F 2+ hr, radiation, ethylene oxide gas (toxic), chemical solutions (bleach 1:100 inactivates HIV; betadine, alcohol). Selection factors: organism, number, equipment type, intended use, available means, **time** [Taylor pp. 681–683, Table 25-3].
- **Supplies/waste**: never reuse a contaminated item on another patient; double bag if outer bag soiled; body-fluid spills cleaned immediately with appropriate germicide; red **BIOHAZARD** bag for liquid/semiliquid OPIM and caked dried blood; paper trays/plastic utensils do **not** prevent transmission (commercial dishwashers suffice) [Taylor p. 685].
- **Box 25-4 CDC highlights not in the slides**: no recapping needles — one-handed scoop or recapping device; safety-engineered devices; puncture-resistant sharps containers; safe injection practices (single-dose vials, new needle/syringe per injection); mask for spinal/epidural procedures; droplet visitors kept 3 ft from infected person; transport droplet/airborne patients only when necessary with surgical mask on the patient [Box 25-4, pp. 687–688].
- **Occupational exposure**: OSHA Bloodborne Pathogens Standard (1991) and Needlestick Safety and Prevention Act (2000) — safer devices, worker involvement in device selection, sharps injury log, education; sharps injuries in ~40% of cases occur in surgery [Taylor p. 688].
- **Isolation** = a protective procedure limiting spread among patients, personnel, and visitors; transmission-based types may be combined but always add to standard precautions [Taylor p. 685] [Taylor p. 686].

NCLEX test traps

- **Trap: "Hand sanitizer is always the best method."** Alcohol rubs are first-line when hands are **not visibly soiled**, but soap and water are required for visibly soiled hands, blood/body fluid contamination, before eating, after the restroom, and for **C. diff, norovirus, Cryptosporidium** [Slide 24] [Taylor p. 677].
- **Trap: "Gloves replace hand hygiene" or reusing gloves between patients/tasks.** Gloves are single-use, changed between tasks and before moving contaminated → clean, and hand hygiene is performed before and after glove removal [Slide 29] [Taylor p. 682].
- **Trap: labeling vague fatigue/malaise/low-grade fever as "incubation."** Incubation = **no symptoms yet**; vague early symptoms = **prodromal**; specific symptoms = **full illness** [Slide 17] [Taylor p. 670].
- **Trap: thinking a fomite is only a piece of trash or that stethoscopes are safe.** Any inanimate object (stethoscope, BP cuff, phone, keyboard) can harbor pathogens; clean/disinfect between patients and never share equipment [Slide 8] [Slide 29] [Taylor p. 668].



Taylor Fig. 25-1: The chain of infection (p. 665)

Module 2 · Ch. 34 Activity & Safe Patient Handling

Ch. 34 Activity & Safe Patient Handling

From the assigned reading

Mechanics, leverage and balance (Figs 34-1, 34-2, 34-7)

- **Levers:** skeletal muscles produce movement by **pulling on bones**; bones act as **levers** and joints act as **fulcrums** (forearm lifting a weight) **[Fig 34-1, Taylor p. 1244]**
- **Balance:** most stable when the **line of gravity passes through a wide base of support**; less stable with a narrow base; **unstable** when the line of gravity falls **outside** the base **[Fig 34-2, Taylor p. 1245]**
- **Correct lifting:** long/strong muscles of arms and legs, object held so line of gravity falls within base of support. **Incorrect:** pull on back muscles and leaning puts line of gravity outside the

base **[Fig 34-7, Taylor p. 1263]**

- Use the **weight of the body to push** an object by falling/rocking **forward** and to **pull** by falling/rocking **backward [Taylor p. 1263]**
- Surface should be **dry and smooth** to decrease friction — rough, wet or soiled surfaces increase friction and effort **[Taylor p. 1263]**
- Flex the knees, **"put on the internal girdle"** (engage core), and come down close to the object to be lifted; **break heavy loads into smaller loads**; take breaks to relax and recover **[Taylor p. 1264]**

Developmental variations — Table 34-3

Age and degree of neuromuscular development markedly influence body proportions, posture, body mass, movements, and reflexes **[Taylor p. 1246]**. For an immobile child, obtain the **history of activity level before the injury** and build an **age-appropriate** plan to maximize function **[Taylor p. 1247]**.

Level	Movement/development highlights	Assessment priorities	Interventions
Infant	Activity/alertness alternate with quiet and sleep; 3 mo raises chest/head prone; ~5 mo head control; 3–6 mo sit/head control; 6–9 mo sits steadily, rolls over, creeps, pulls to stand, improved hand–eye; 9–12 mo progressing to unassisted walking, picks up small objects	Milestones at 3–6, 6–9, 9–12 months	Encourage parents to examine baby; respond to concerns about minor variations; emphasize individual variation; account for prematurity **[Table 34-3, Taylor pp. 1246–1247]**
Toddler	15 mo walk unassisted; 18 mo run; 2 yr jump; 3 yr stack blocks, simple puzzles, dress self	Walking/running/jumping; small-muscle coordination (dress, wash hands, brush teeth); distinguish slow developers from developmental lags	Teach safe environment; enthusiastically praise mastery; set limits so toddler does not overextend **[Table 34-3, Taylor pp. 1247–1248]**
Child	Age 4: stairs, walk backward, hop on one foot; age 5: skip, jump rope, jump off several steps; manipulates writing materials	Developmental charts for gross/fine motor; activity level and types of physically exerting play	Attitudes about body and exercise are developed in this period; reinforce safety teaching at each stage **[Table 34-3, Taylor p. 1248]**
Adolescent	Growth spurt, secondary sex characteristics; fitness can mean boundless energy; inactivity may start lifelong unhealthy behavior	Activity/exercise type; safety of recreational choices; **screen for scoliosis**; muscle mass, tone, strength, joint mobility	Encourage exercise, limit sedentary hobbies; lifestyle counseling; caution about "pushing too hard" **[Table 34-3, Taylor p. 1248]**

Adult	Erect sitting/standing, balanced coordinated movement; **pregnancy shifts center of gravity**; activity levels vary greatly	Balance of activity vs rest; lifestyle factors/illnesses interfering with mobility or ADLs	Fitness counseling; clarify exercise misconceptions; design/monitor safe exercise programs; special care for mobility alterations **[Table 34-3, Taylor p. 1248]**

- **IADLs** (higher-level function) = housekeeping, meal preparation, management of finances, transportation — distinct from ADLs (eating, bathing, dressing, toileting) **[Taylor p. 1247]**

Safe handling details & dementia — Box 34-1

- Safe handling requires **assessment criteria, screening/algorithms, and proper use of equipment**; keep the patient in **good alignment** and protect from injury **[Taylor p. 1264]**
- Injury-prevention factors: **user training, device wear and tear, suitability of equipment, correct operation [Taylor p. 1264]**
- **Eliminate or reduce unnecessary tasks** to reduce injury risk and increase self-esteem and mobility; **patient engagement** during handling prevents adverse events **[Taylor p. 1264]**
- Use a **standardized screening tool** for consistency and appropriate SPHM equipment selection **[Taylor p. 1264]**

Safe handling of patients with dementia [Box 34-1, Taylor p. 1263] — communication problems and weakness make handling challenging:

- **Face the patient**; speak **slowly**; use **clear, short sentences**; call the patient **by name**; use a **calm, reassuring tone**; **allow time for a response**.
- Give **simple, one-step instructions**; repeat verbal cues/prompts as needed when thought processes are delayed.
- Determine if the patient has **receptive aphasia** (inability to understand) — this causes apparent noncompliance.
- **Phrase instructions positively:** say "Stand up" until the chair is positioned — not "Don't sit down" (the "don't" may not register; patient sits too early).
- Ask **one question at a time**, allow an answer, repeat if necessary.
- Allow focus on the task; **avoid correcting the process** unless it would be dangerous.
- **Incorporate established habits/customs** into desired activities (e.g., a patient used to evening baths may resist a morning shower; a patient may resist getting out of the "wrong side" of the bed).

Patient positions & complications to prevent — Table 34-6

- When the patient cannot self-reposition, use a **repositioning schedule**; frequency should be **evaluated, reevaluated, and tailored to the individual** — document rotation of positions **[Taylor p. 1269]**

Position	Definition / key points	Complication to prevent	Preventive action
Fowler's (head of bed **45–60°**) — promotes cardiac/respiratory function; position of choice for eating, conversation,	Flexion contracture of the neck	Head rests against mattress or small pillow only	

urinary/intestinal elimination. **High-Fowler's = 90°** (pillow on bedside table → maximal lung expansion); **low/semi-Fowler's = 30°**. Buttocks bear main weight; heels, sacrum, scapulae at risk for breakdown. **Avoid semi-Fowler's in patients at risk for skin integrity alterations** (shearing over sacrum)	Exaggerated curvature of the spine	Firm back support; elevation angle starts at the hips	
	Dislocation of the shoulder; wrist flexion contracture; edema of the hand	Forearms on pillows (no pull on shoulders); hand in natural alignment with forearm, slightly elevated relative to the elbow; hand–wrist splints for finger flexion/thumb abduction	
	Impaired lower extremity circulation & knee contracture; pressure on heels	Elevate knees briefly only; pillows under lower legs from below knees to ankles; avoid popliteal pressure and the knee gatch	
	External rotation of the hips	Trochanter roll	
	Footdrop	Feet supported in dorsiflexion; footboard or high-top sneakers	
	Source rows: **[Table 34-6, Taylor pp. 1270–1271]**		
Supine / dorsal recumbent (flat on back; head and shoulders slightly elevated with a pillow **unless contraindicated**, e.g., spinal anesthesia or spinal surgery)	Exaggerated spinal curvature; flexion of the hips	Firm supportive mattress; bed board if needed	
	Flexion contracture of the neck	Pillows under upper shoulders, neck, and head	
	Internal rotation of shoulders / extension of elbows (hunched shoulders); flexion of lumbar curvature	Pillows or arm supports under forearms, arms at sides, forearms pronated slightly; rolled towel or small pillow under lumbar curve if needed	
	Finger extension/thumb abduction; external rotation of the femurs; hyperextension of the knees	Hand–wrist splints if appropriate; sandbags or trochanter roll along hips and upper thighs; pillow under lower legs from below the knees **[Table 34-6, Taylor p. 1271]**	

Side-lying / lateral	Weight borne by **lateral aspect of lower scapula and lower ilium**; comfortable for sleep; relieves pressure on scapulae, sacrum, heels; support pillows required	See Table 34-6 for support pillows	**[Taylor p. 1270]**
Oblique / 30° side-lying	Hip of top leg flexed **30°** (not 90°), knee flexed **35°**, calf slightly behind midline; pillows behind back and under calf of top leg	Significantly **less pressure on trochanter and sacrococcygeal areas** vs. 90° side-lying	**[Fig 34-20, Taylor p. 1270]**
Sims'	Side-lying variation: lower arm behind patient, upper arm flexed at shoulder and elbow	Different pressure points — weight on **anterior humerus, clavicle, ilium**	**[Taylor p. 1270]**
Prone	On abdomen, head turned to side; shoulders/head/neck erect; hips extended; prevents hip/knee flexion contractures	**Lordosis** (contraindicated in **spinal problems**); **plantar flexion / footdrop** from gravity on feet	Position legs and feet carefully (see Table 34-6) **[Taylor p. 1270]**

- **VTE prophylaxis:** graduated compression stockings and pneumatic compression devices are **passive** interventions for venous stasis, thrombophlebitis, and venous thrombosis; may be used alone or with **pharmacologic** prophylaxis **[Taylor p. 1270]**

NCLEX test traps

- **Twisting the trunk** to move/reposition a patient is always wrong — **face the direction of movement**; move with feet, not the waist. **Pull** is a distractor: **push** with the whole body, arms close.
- **Bending at the waist with straight legs** ("lift with your back") is a classic wrong answer — flex the knees and hips, keep the object close so legs/arms bear the load; back muscles are weaker and injury-prone.
- **Narrow base of support** or standing far from the object = unstable (line of gravity outside base). Correct: feet **shoulder-width**, knees/hips flexed, object close to COG.
- **Degree confusion:** Fowler's = **45–60°**; high-Fowler's = **90°**; low/semi-Fowler's = **30°**. Test questions will swap these.
- **Picking semi-Fowler's for a patient at risk for skin breakdown** — avoid it; even a few centimeters of head elevation increases **sacral shearing**. Also remember Fowler's pressure points: heels, sacrum, scapulae (and buttocks bear weight).
- **Prone position** is a trap for patients with **spinal problems** (gravity produces lumbar lordosis) and requires foot positioning/footboard to prevent **plantar flexion/footdrop**. Trochanter rolls prevent **external hip rotation**; footboards prevent footdrop.
- **"Reposition every 2 hours for every patient"** is too rigid — frequency is **individualized**, reevaluated, and documented on a schedule when the patient cannot move independently.
- **Dementia handling distractors:** long sentences, "Don't…" (negative) instructions, quizzing multiple questions at a time, rushing, correcting the process mid-task. Correct: face the patient,

name, slow calm speech, short **positive** one-step commands, allow response time, work **with** established habits; check for **receptive aphasia**.
- **Manual lifting without assistive devices** (or "just get more staff and lift") is unsafe — use friction-reducing sheets, gait belts, standing/lateral assist devices, powered full-body lifts; screen first, get enough staff, lock wheels, elevate bed, give analgesia before movement.
- **Do not let the patient "do nothing"** — encourage the patient to assist within capability to promote independence and self-esteem; likewise, **ADL ≠ IADL** (finances, transportation, meal prep, housekeeping are IADLs).

Modules 2–3 · Ch. 26 Vital Signs

Ch. 26 Vital Signs

From the assigned reading (Taylor — details the slides do not cover)

General principles

- VS assessment may be delegated to assistive personnel when the patient is stable, but the **nurse** is responsible for accuracy of data, interpretation, and reporting abnormal findings; validate unexpected findings and assess further. [Taylor p. 717]
- Frequency: hospitalized patients with elevated temps, high/low BP, pulse rate/rhythm changes, or respiratory difficulty are assessed **at 4-hour intervals at minimum**; long-term care requires monthly VS (daily for Medicare A skilled residents); critical care uses continuous monitoring. [Taylor p. 717]
- VS plus oxygen saturation are identified as the best indicators of cardiopulmonary arrest, unplanned ICU admission, and unexpected death; trends over time aid early detection of deterioration. [Taylor p. 716]
- Normal values vary across the life span (Table 26-1); always compare to the patient’s own baseline. [Taylor Table 26-1]

Temperature

- Normal body temperature ranges **96.7–100.5 °F (35.9–38 °C)** depending on route. [Taylor p. 718]
- Heat production: metabolism (primary), epinephrine/norepinephrine (rapid), thyroid hormone (slow), shivering, piloerection (“goose bumps” reduces surface area for heat loss), physical exertion. [Taylor p. 719]
- Heat loss: skin is the primary site; **arteriovenous shunts** open to dissipate or close to retain heat (sympathetic control). Mechanisms of heat transfer: **radiation, convection, evaporation, conduction**. [Taylor p. 719; Taylor Table 26-2]
- Circadian variation can be up to 1.0 °F (0.5 °C)/day; peak elevation occurs **3–6 PM**; progesterone at ovulation raises a woman’s temp 0.5–1.0 °F. [Taylor p. 719]
- Fever results from an upward displacement of the hypothalamic set point caused by **pyrogens** (bacteria, bacterial products, viruses, chemicals from tissue injury — MI, PE, cancer, trauma, surgery); rising mechanisms are shivering, piloerection, vasoconstriction, increased metabolism. [Taylor p. 720]
- Fever patterns (Box 26-2): **intermittent** (returns to normal ≥ once/24 h), **remittent** (never normal, fluctuates), **sustained/continuous** (above normal with minimal variation), **relapsing/recurrent** (normal for ≥1 day between episodes). [Taylor Box 26-2]

- **Hyperthermia differs from fever:** hypothalamic set point is NOT changed — cooling mechanisms are simply overwhelmed. **Neurogenic fever** (hypothalamic damage) does not respond to antipyretics. **FUO** = ≥ 38.3 °C (101 °F) for ≥ 3 weeks without identified cause. [Taylor p. 720]
- Fever complications: febrile seizures (infants/children), confusion/delirium (older adults), fever blisters, fluid/electrolyte/acid–base imbalances. Older adults run a lower baseline — fever may be a **late** sign and only slightly above normal even in serious infection; mild elevation in infants < 3 months may signal serious infection. [Taylor p. 720–721]
- Antipyretics (aspirin, ibuprofen, acetaminophen) reset the set point; **routine antipyretic administration is usually avoided**; **no aspirin or aspirin-containing products for anyone < 19 years** with fever/flu-like illness (Reye syndrome). Non-pharm: cool sponge baths, cool packs, cooling (hypothermia) blankets, oral fluids, simple carbohydrates. [Taylor p. 721]
- **Hypothermia = core temp below 35 °C (95 °F)**; risks: unprotected cold exposure, older adults/disabled, alcoholism, malnutrition, hypothyroidism, perioperative patients, neonates. **Therapeutic hypothermia** (purposeful cooling) improves neurologic outcomes after brain injury, cardiac arrest, some surgeries. [Taylor p. 721]
- Hypothermia blanket (Guidelines 26-1): protect skin with lanolin/cold cream, cover pad with a sheet, insert rectal/esophageal/axillary probe, elevate scrotum off the pad, turn q30 min–1 h, monitor VS/neuro checks (often q15 min), watch for shivering, discontinue ~1 degree above desired temp. [Taylor p. 722–723]
- Equipment: mercury glass thermometers are not used in facilities; shared/nondisposable thermometers require cleaning/disinfection (cross-contamination risk); noncontact infrared thermometers are for **fever screening, not ongoing monitoring**. [Taylor p. 723–724]
- **Always document the site**; if no site is documented, it is assumed to be **oral**. Comparing two different sites is a way to double-check an unusual reading. [Taylor p. 725]
- Oral: contraindicated in children < 6 years, unable to follow directions, oral mucosa inflammation/surgery, confused/comatose, mouth breathing, seizure risk, and **patients on oxygen by mask**; wait 15–30 min after hot/cold food/fluids or smoking. Normal oral 96.6–99.5 °F. [Taylor p. 725]
- Tympanic: do not use with ear drainage, ear pain, ear infection, or tympanic scarring — use the **opposite ear**; cerumen affects readings. Normal tympanic 98.2–100.9 °F. [Taylor p. 725]
- Temporal artery: head coverings/hair/wigs insulate → **falsely high**; measure only the exposed side if lying on one side; do not measure over scar tissue, lesions, abrasions; diaphoresis impairs accuracy. Normal 98.7–100.5 °F. [Taylor p. 725–726]
- Axillary: less accurate — do not use when accuracy is required; most common site in the **neonate**; hold arm to the side until complete. Normal 95.6–98.5 °F. [Taylor p. 726]
- Rectal: one of the most accurate/core-reflective routes, BUT contraindicated in **newborns, children with diarrhea, recent rectal/anal/vaginal/prostate surgery, rectal disease, neutropenia, thrombocytopenia, certain neurologic disorders (spinal cord injury)**; vagal stimulation may slow the heart rate (avoid in some cardiac patients). Normal 97.4–100.5 °F. [Taylor p. 726]

Pulse

- Pulse is regulated by the autonomic nervous system through the **SA node**; vagal (parasympathetic) stimulation slows HR; sympathetic stimulation increases HR and force of contraction. [Taylor p. 727]
- Factors affecting pulse (Table 26-4): age (older adults may have decreased rate), biologic sex (women slightly higher), physical activity (athletes may be below normal), body temperature

(fever ↑, hypothermia ↓), stress/pain/fear/anxiety, medications (↑ or ↓), disease/health conditions (COPD, pneumonia, sepsis, PVD, heart failure). [Taylor Table 26-4]

- Tachycardia causes (Box 26-3): ↓ BP (blood loss, shock, trauma), elevated temperature, poor oxygenation (chronic pulmonary disease, anemia), exercise, hyperthyroidism, pain, strong emotions, some drugs (atropine, epinephrine). Bradycardia causes: CAD/MI, decreased temperature, hypoxemia/OSA, delirium, hypothyroidism, increased ICP/CVA, vagal stimulation (ocular pressure, vomiting), drugs (digitalis, antiarrhythmics). [Taylor Box 26-3]
- Pulse amplitude grading (Table 26-5): **0** absent; **+1** diminished; **+2** normal/brisk; **+3** bounding. Bounding with early septic shock, exercise, fever, fluid volume excess, anxiety; weak/thready with heart failure, hypovolemia, shock, hypernatremia, arrhythmias. [Taylor Table 26-5, p. 728]
- **Pulse elasticity** (a 4th characteristic): normal artery feels smooth, straight, springy; loses elasticity with age. [Taylor p. 728]
- Radial pulse is hard to palpate accurately in children < 2 years — **auscultate apical pulse for 1 full minute**. Carotid: palpate **one side at a time** (cerebrovascular safety), used in shock/cardiac arrest; **brachial** is used for infants in emergencies. [Taylor p. 730]
- Apical pulse site: **5th intercostal space, left midclavicular line**; alternative Erb's point (2nd–3rd ICS left sternal border); count a full minute. Required when peripheral pulse is irregular, weak, or very rapid, and **before giving medications that alter heart rate/rhythm**. [Taylor p. 730–731]
- **Apical–radial pulse (2 nurses, 1 shared watch, both count 1 full minute)**: the difference is the **pulse deficit** — indicates not all beats reach the periphery/insufficient cardiac force → decreased tissue perfusion. [Taylor p. 731–732]
- Stethoscope: **diaphragm** = high-frequency sounds (lungs); **bell** = low-frequency sounds (heart/vessels). Doppler ultrasound used for pulses too weak to palpate (and for systolic BP estimate). [Taylor p. 729, 742]

Respirations

- Respiration = **ventilation** (movement of gases in/out), **diffusion** (gas exchange alveoli ↔ blood), **perfusion** (blood ↔ tissue cells). Only ventilation is measured as a VS. Normal unlabored breathing = **eupnea**; ~1 respiration per 4 heartbeats in healthy people. [Taylor p. 731, 733]
- Increased CO_2 is the most powerful respiratory stimulant; cerebral cortex gives voluntary control. [Taylor p. 732]
- Factors (Box 26-4): age (newborn 30–60 → adult 12–20), exercise, acid–base imbalance (acidosis → hyperventilation), brain lesions (Cheyne–Stokes), altitude, respiratory disease/smoking, anemia, anxiety (sighing respirations), medications (narcotics/sedatives/anesthetics slow rate & depth; amphetamines/cocaine increase), acute pain (↑ rate, ↓ depth). [Taylor Box 26-4]
- Abnormal patterns (Table 26-6): **tachypnea > 24 breaths/min, shallow**; **bradypnea < 10 breaths/min, regular**; **hyperventilation** = increased rate and depth (fear, exercise, DKA/Kussmaul); **hypoventilation** = decreased rate and depth, irregular (narcotic/anesthetic overdose); **Cheyne–Stokes** = alternating deep rapid breathing with apnea (drug overdose, HF, increased ICP, renal failure); **Biot's** = varying depth/rate with periods of apnea, irregular (meningitis, severe brain damage). [Taylor Table 26-6]
- **Apnea** without intervention → death within a few minutes. **Orthopnea** = breathes more easily upright; SOB when lying flat (pulmonary edema, heart disease, COPD). **Dyspnea** is perceived only by the person experiencing it. [Taylor p. 734]

Blood pressure

- **Pulse pressure = systolic − diastolic** (e.g., 120/80 → pulse pressure 40 mm Hg). Cardiac output and peripheral resistance determine systolic and diastolic pressures. [Taylor p. 734]

- Short-term regulation: neural (cardiovascular center, vagus, sympathetic) + humoral (epinephrine, renin–angiotensin–aldosterone → vasoconstriction, Na/water retention; ADH/vasopressin). Long-term: renal regulation of extracellular fluid volume. **Baroreceptors** (aortic arch, carotid) sense vessel stretch. [Taylor p. 734–735]
- Factors in healthy people (Box 26-5): age (↓ arterial elasticity), circadian rhythm (lowest on arising; rises 5–10 mm Hg by late afternoon), biologic sex (women lower until menopause), food intake (↑ after eating), exercise (↑ systolic), weight (↑ with obesity), emotional state (anger, fear, excitement, pain ↑), body position (lower supine/prone than sitting/standing), race (hypertension more prevalent/severe in African American adults), drugs (oral contraceptives ↑). [Taylor Box 26-5]
- A rise or fall of **20–30 mm Hg from a person's own baseline is significant** even if within the normal range. A single elevated reading is not necessarily significant; hypertension is diagnosed on the **average of 2+ readings on 2+ occasions**. [Taylor p. 735]

Category (adults ≥ 18)	Systolic		Diastolic
Normal	< 120	**and**	< 80
Elevated	120–129	**and**	< 80
Hypertension Stage 1	130–139	**or**	80–89
Hypertension Stage 2	≥ 140	**or**	≥ 90
Hypertensive crisis	> 180	**and/or**	> 120

[Taylor Table 26-7]

- Hypertension is a major risk factor for CVA, MI, heart failure, PVD, abdominal aortic aneurysm ("silent killer"). Treatment: lifestyle changes (weight loss, reduced sodium/saturated fat, ↑ fruits/vegetables/grains, limit alcohol, stress management, regular activity) plus antihypertensives (diuretics, ACE inhibitors, ARBs, vasodilators/CCBs, beta-blockers). [Taylor p. 736–737]
- **Hypotension = BP lower than 90/60 mm Hg.** Concern when symptomatic: dizziness, tachycardia, pallor, syncope, diaphoresis, blurred vision, nausea, confusion — report immediately. [Taylor p. 737]
- **Orthostatic (postural) hypotension:** sustained drop of **≥ 20 mm Hg systolic or ≥ 10 mm Hg diastolic within 3 minutes of standing**; also delayed type (> 3 min) and initial type (transient ≥ 40 systolic/≥ 20 diastolic within 15 sec). Causes: dehydration/blood loss, neurologic/cardiovascular/endocrine problems, medications. **Older adults at increased fall risk.** [Taylor p. 737–738]
- Orthostatic assessment (Guidelines 26-5): supine 3–10 min → baseline BP/pulse; sit on edge with legs dangling 1–3 min → BP/pulse; stand, measure at 30 sec, 60 sec, and up to 3 min. Return supine immediately if dizziness/pallor/diaphoresis/syncope; do not proceed to standing if symptomatic when sitting. Dangle before standing after bed rest. [Taylor p. 738]

Cuff selection & measurement technique

- Cuff bladder **length 75%–100%** of arm circumference, **width 37%–50%** (ratio 2:1); sizes: small adult 12 × 22 cm (arm 22–26 cm), adult 16 × 30 (27–34 cm), large adult 16 × 36 (35–44 cm), thigh 16 × 42 (45–52 cm). **Incorrect cuff size is the most frequent BP error**. Cuff too narrow → falsely high; too wide → falsely low. Cuff taut: one finger fits easily at top/bottom, two fingers very snug. [Taylor Table 26-8, p. 739–740]

- Patient prep: quiet environment; **no caffeine, exercise, or smoking for 30 min before**; no talking or moving; back supported (no back support ↑ systolic up to 15 mm Hg, diastolic up to 6); **legs uncrossed** (crossed ↑ systolic up to 8 mm Hg); arm supported at heart level (midpoint of sternum). Supine readings may run up to 10 mm Hg systolic/5 mm Hg diastolic higher than seated. [Taylor p. 740]
- Errors (Box 26-6): noncalibrated/nonvalidated device; wrong cuff size; patient movement/talking; incorrect cuff placement/orientation; incorrect patient or limb positioning; poor technique; **inflating too rapidly**; **deflating faster than 2–3 mm Hg/second**; environmental noise; no rest period; incorrect interpretation; relying on a single-occasion reading. [Taylor Box 26-6]
- Methods: auscultatory (sphygmomanometer + stethoscope), **oscillometric** (automated, now common; devices require calibration/validated use), Doppler (estimate of **systolic only**, when sounds inaudible), direct arterial line (ICU), ambulatory (ABPM) and home (HBPM) monitoring. [Taylor p. 741–743]
- BP sites: brachial and popliteal most common; forearm/radial for very large or obese arms; wrist or ankle when no cuff fits. [Taylor p. 743]

Korotkoff sounds (auscultatory method)

Phase	Description
I	First appearance of faint but clear **tapping** sounds that gradually increase in intensity — **first tapping sound = systolic pressure**
II	**Muffled or swishing** sounds; may temporarily disappear — the **auscultatory gap** (up to 40 mm Hg). Missing it → underestimating systolic or overestimating diastolic
III	Distinct, **loud** sounds as blood flows relatively freely through an increasingly open artery
IV	Distinct, abrupt **muffling** with a soft, blowing quality
V	**Last sound heard before loss of all sounds = diastolic pressure**

Read to the nearest even number (nearest 2 mm Hg). If sounds are heard all the way to zero (pregnancy, AV fistula), record Phase IV as well (e.g., 112/62/0). [Taylor Table 26-9, p. 742]

Pain & pulse oximetry (context)

- Pain assessment is routinely included with VS (discussed in depth in Ch. 36); use PQRST for verbal, aware patients. [Taylor p. 716; Slide 24]
- Pulse oximetry is the noninvasive measurement of arterial oxyhemoglobin saturation (SpO2) — detailed in Ch. 40; it complements, but does not replace, full respiratory assessment and ABG monitoring. [Taylor p. 716, 734]

⚠ **Conflict:** Elevated/prehypertension category — Slide 20 lists “systolic 120–129 and diastolic of 80,” while Taylor Table 26-7 defines **Elevated = systolic 120–129 AND diastolic < 80**. **Go with the slide wording for her exam**, but know the AHA/Taylor table for NCLEX. [Slide 20; Taylor Table 26-7]

⚠ **Conflict:** Hypertensive crisis threshold — Slide 20 states “> 180” (systolic only), while Taylor Table 26-7 states **> 180 systolic and/or > 120 diastolic**. Go with the slides for her exam. [Slide 20; Taylor Table 26-7]

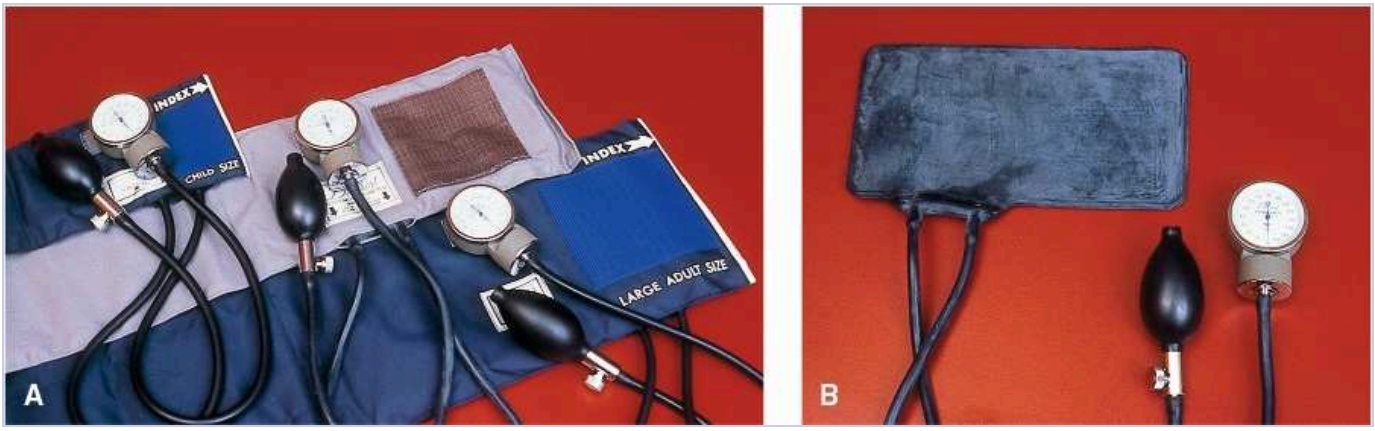
⚠ **Conflict:** Cuff bladder coverage — Slide 21 states the bladder should cover “2/3 of the arm,” while Taylor Table 26-8 recommends bladder length of **75%–100% of arm circumference** (width 37%–50%, ratio 2:1). Go with the slides for her exam. [Slide 21; Taylor Table 26-8]

NCLEX test traps

- **Thumb vs. fingers:** never palpate a pulse with your thumb (it has its own pulse) — use the pads of the 2nd, 3rd, or 4th fingers. Also: 30 sec × 2 is acceptable **only for a regular** rhythm; an irregular rhythm requires a full 60-second count (radial, plus apical for a pulse deficit).
- **BP category logic (and vs. or):** Normal and Elevated require the systolic **AND** diastolic criteria to be met; Hypertension Stage 1/2 are met by the systolic **OR** diastolic value (e.g., 118/92 = Stage 1, not "normal").
- **Cuff size errors:** cuff **too narrow/too small → falsely HIGH**; cuff **too wide/too large → falsely LOW**. Size by **arm circumference, not the patient's age**. Too loose = falsely high.
- **Arm position errors:** arm **above** heart level → falsely low; arm **below** heart level → falsely high. Unsupported back, crossed legs, talking, recent caffeine/smoking/exercise, and recent activity all raise readings. No back support can raise systolic up to 15 mm Hg.
- **Korotkoff phases:** Phase I = systolic; **Phase V (not IV) = diastolic**. The **auscultatory gap** (Phase II) can span up to 40 mm Hg — missing it underestimates systolic or overestimates diastolic. Deflate at 2–3 mm Hg/second and read to the nearest 2 mm Hg. In pregnancy or with AV fistulas, sounds may persist to zero — document Phase IV too.
- **Limb selection:** avoid the arm with a PICC line, an AV fistula, or a mastectomy; use the other arm, forearm/wrist, or ankle instead.
- **Respirations counted "out loud":** tell the patient you are counting the pulse, keep the hand on the wrist, and observe chest rise/fall covertly — a patient who knows respirations are being counted will change the rate.
- **Temperature site math and contraindications:** axillary = oral − 0.5 °C; rectal = oral + 0.5 °C. Oral is contraindicated in children < 6 years (bite risk), unconscious/confused patients, mouth breathers, after hot/cold intake or smoking, and in patients on an oxygen mask. Rectal is contraindicated in newborns, diarrhea, thrombocytopenia/neutropenia, recent rectal/prostate surgery, and some cardiac/neurologic patients. Tympanic is distorted by cerumen, otitis media, and ear drainage (use the opposite ear).
- **Fever numbers:** > 40 °C (

AGE	TEMPERATURE °F °C	PULSE beats/min	RESPIRATIONS breaths/min	BLOOD PRESSURE mm Hg
Newborns	97.2–99.9 36.2–37.7	95–170	30–60	60–70/40
Infants	96–99.7 35.6–37.6	85–170	30–50	85/37
Toddler	96–99 35.6–37.2	70–150	20–40	88/42
Child	96–99 35.6–37.2	65–130	15–25	95/57
Adolescent	96.4–99.5 35.8–37.5	60–115	12–20	102/60
Adult	96.4–99.5 35.8–37.5	60–100	12–20	<120/80

Taylor Table 26-1: Age-related variations in normal vital signs (p. 718)



Taylor Table 26-9: Korotkoff sounds

Module 4 · Ch. 8 Communication

Ch. 8 Communication

From the assigned reading

- **Process refinements:** A patient need stimulates communication. Sender and receiver participate reciprocally and simultaneously. Prior knowledge, experience, feelings, and sociocultural background affect interpretation. **Noise** is anything that distorts a message, including television, pain, or discomfort. Feedback helps verify intended meaning; choose messages requiring minimal decoding effort. [Taylor p. 173] [Taylor p. 175]
- **Verbal/nonverbal distinctions:** Taylor includes **written and spoken words** as verbal communication. Crying, moaning, gasping, and sighing are oral but **nonverbal**. Silence has multiple meanings, including thought, comfort, fear, anger, stoicism, and culturally expected pauses. [Taylor p. 175] [Taylor p. 178] [Taylor p. 193]
- **Cultural interpretation:** Eye contact can convey respect or be experienced as intrusive/confrontational. Gestures vary culturally. Monitor your own expression: disgust or impatience can damage the patient's self-image and trust. Behavior is a cue for assessment, not proof of a specific feeling. [Taylor p. 177]
- **Social media:** Follow HIPAA, professional boundaries, and school/facility policy. Removing names does not guarantee anonymity; room numbers or diagnoses can identify patients. Posts may be redistributed and remain accessible after deletion. Privacy settings do not remove accountability. Report harmful content; separate personal/professional information and help develop online-conduct policies. Consequences may include job loss, board discipline, loss of licensure, and civil/criminal penalties. [Taylor p. 178] [Taylor p. 179] [Box 8-2]
- **E-mail/text:** Use approved safeguards, concise wording, and no text abbreviations. Patient authorization may be required; patient e-mails become part of the health record. **Telehealth:** Professional standards remain unchanged; dress professionally, reduce noise/clutter, ensure privacy, prevent interruptions, and use visible attentive cues. [Taylor p. 179]
- **Positive self-talk and groups:** Self-talk can improve preparation and confidence, not only cause harm. Small groups include meetings, conferences, teaching, and support groups; organizational communication advances institutional goals. Effective groups share goals, trust, two-way communication, problem-solving, responsibility, distributed leadership, and appropriate

power use; ineffective groups show alienation, domination, unresolved problems, and power abuse. [Taylor p. 179] [Taylor p. 180] [Table 8-1]

- **Development/roles:** Match language to cognitive development, health literacy, and individual needs. Do not assume a health professional needs no teaching or that an uncomplaining patient has no needs. Territoriality is a person's claim to personal space; home versus institutional settings can change communication. [Taylor p. 180] [Taylor p. 181] [Taylor p. 182]
- **Three relationship phases:**
  - **Orientation:** Introduce names, clarify roles, build trust, orient to the facility, and agree on goals, meeting location/frequency/length, and relationship duration. [Taylor p. 187] [Box 8-3]
  - **Working/identification and exploration:** Usually longest; active participation, expression of concerns, mutually agreed goals, assistance with daily living, teaching, counseling, and implementation of care. [Taylor p. 187]
  - **Termination/resolution:** Review goals and progress, acknowledge remaining needs, invite feelings about ending, and prepare for the next caregiver/setting. Can occur at shift change, discharge, or nurse departure. Phases may overlap rather than follow a rigid sequence. [Taylor p. 187] [Taylor p. 188]
- **Culturally responsive care:** Examine personal biases; use the patient as an information source; explore individual meanings of health/illness and differences in expectations. Adapt care, pacing, and listening behaviors; validate meaning rather than assuming understanding. Respect reluctance about sensitive topics. Simple repeated wording, pictures, gestures, and appropriate language aids may support communication; interpreters translate meaning, not merely words. [Box 8-4]
- **Rapport builders:** Rapport is mutual trust. Set an interaction objective while staying flexible; provide comfort, privacy, confidentiality, and patient-focused pacing. Explain who will receive information and be honest about available time. Competence includes technical, cognitive, interpersonal, and ethical/legal skills; evaluate your strengths and limitations. [Taylor p. 190] [Taylor p. 191]
- **Additional interviewing skills:** Explain the interview's purpose and plan follow-up. **Sequencing** places events in time and explores possible cause/effect; **directing** returns to an earlier topic or develops a related aspect. Excessive clarification/validation can imply inattentiveness. Listen for repeated themes, avoided topics, gaps, and inconsistencies; think before responding. **Semantics** is the meaning of words. [Taylor p. 192] [Taylor p. 193] [Taylor p. 194] [Taylor p. 195]
- **Assertiveness:** Open, honest, direct, nondefensive communication focused on the **issue, not the person**; use clear "I" statements and respectful posture/tone. Ask for help, admit mistakes, and accept responsibility. Aggression violates others' rights; nonassertiveness/avoidance fails to communicate needs clearly. Taylor references **CUS** as a safety-speaking-up method, but its expansion is not provided in this excerpt. [Taylor p. 195] [Taylor p. 196] [Table 8-2]
- **Additional barriers:** Avoid patronizing names such as "sweetie," stereotypes, intimidating "how" questions, and premature probing. If false reassurance slips out, acknowledge it, apologize, and return to therapeutic communication. [Taylor p. 196] [Taylor p. 197] [Taylor p. 198]
- **Incivility/bullying:** Incivility includes rude/intimidating behavior and failure to assist or share needed information. Bullying is persistent intimidation; covert forms include exclusion, withheld information, rumors, and eye-rolling; overt forms include humiliation and scapegoating. Nurse-to-nurse hostility is also called horizontal/lateral violence. Consequences include distress, burnout, turnover, teamwork failure, and patient-care errors. [Taylor p. 199]

- **Responding to disruption:** Remain calm/professional, address behavior assertively, speak privately when appropriate, and use management/reporting channels. Document facts. Organizations need education, accountability, zero-tolerance policies, reporter protection, leadership role models, and surveillance/reporting systems. Violence is **not an expected part of nursing**. Communication is a first defense, supported by organizational safety measures. Education and conflict-management training can improve difficult conversations. [Taylor p. 199] [Taylor p. 200] [Taylor p. 201]
- **Impaired verbal communication:** Decreased, delayed, or absent ability to use speech. Adapt calmly and individually; use supportive gestures, clarification, and validation. Avoid **elderspeak**—diminutives, exaggerated pitch/intonation, and language implying incompetence. [Taylor p. 201]
- **Visual impairment:** Announce presence/name; speak normally; explain before touching; describe surroundings and sounds; state when leaving; keep call device within reach and glasses/contacts usable. [Box 8-6]
- **Deaf/hard of hearing:** Gain attention, face the patient, use simple sentences and natural speech; keep mouth visible and do not chew gum. Use appropriate demonstration, sign/finger spelling, or writing; check hearing-aid function and placement. [Box 8-6]
- **Physical speech barrier:** Select usable communication methods—blinks/squeezes for yes/no, writing, word/letter/picture boards, flash cards, or sign language. Ensure all caregivers/family understand the method; allow time, reinforce effort, and provide a reliable assistance signal. **Source limit:** Box 8-6 ends at the cognitive-impairment heading; no missing recommendations are inferred. [Box 8-6]
- ⚠ Conflict: **Slide 7** lists verbal communication as spoken; **Taylor p. 175** includes spoken and written words. Use the slide's classification for her exam; recognize Taylor's broader definition.
- ⚠ Conflict: **Slide 33** lists closed-ended questions and advice among techniques to avoid; **Taylor pp. 194, 197–198** permits focused questions for specific data and requested advice within expertise. For her exam, follow the slide's barrier classification; the activity deck also explicitly supports neutral focused assessment questions [activity Slide 17].
- ⚠ Conflict: **Slide 25** labels goal-review prompts "reflective"; **Taylor p. 195** defines reflection as repeating content or describing feelings. Use the instructor's labels on her exam while recognizing both forms in the supplied materials.
- ⚠ Conflict: **Slide 25** explicitly requires consent for touch beyond hand/shoulder contact; **Taylor p. 194** recommends asking before touch generally. The later **activity Slide 19** also says to ask before offering a hand or other touch; follow that explicit slide guidance.

NCLEX test traps

- **Do not automatically fill silence:** Attentive quiet may permit processing and expression; withdrawal or ignoring the patient is different. [Slide 25; Taylor p. 193]
- **Nonverbal cues are not a diagnosis:** Investigate mismatches without accusing the patient or discarding self-report; account for cultural variation. [Activity Slide 20; Taylor p. 177]
- **Professional boundaries persist online:** No patient name, a private account, or deleting a post does not guarantee confidentiality. Assertiveness is also not aggression; address safety concerns without threats or humiliation. [Taylor pp. 178–179] [Taylor pp. 195–196]
- **Adapt without infantilizing:** Speech impairment does not justify assuming incompetence. Allow time, provide usable aids, preserve dignity, and avoid elderspeak. Termination includes shift change—not only discharge. [Taylor p. 188] [Taylor p. 201] [Box 8-6]

Lab skills: book-based test traps

From the assigned reading

The provided source (Weekly Skills & TracPrac Breakdown, Pages 1–3) does not include Taylor textbook excerpts or page/box/table markers. If Taylor reading assignments are distributed separately, verify key points against the ATI skill checklists cited above and the instructor's stated priorities.

NCLEX test traps

- **Hand hygiene:** Wrong answer picks alcohol-based rub when soap & water is required (visible soil, *C. difficile*, before eating, after toileting). Don't confuse the two.
- **PPE order:** Trap answers reverse don/doff sequence. Don Gown→Mask/Goggles→Gloves; Doff Gloves→Goggles→Gown→Mask. Removing the mask first contaminates everything below it.
- **Standard vs. transmission-based:** Standard precautions apply to *every* patient. Answer choices that limit precautions to "known infections only" are wrong.
- **Body mechanics:** Distractors say "lock your knees and bend at the waist" — the correct answer is always **wide stance, bend knees, keep load close**.
- **Temperature:** Trap lists the same number for every route. Remember axillary < oral < rectal/tympanic; temporal is approximate.
- **Pulse oximetry:** Wrong answers ignore factors that **falsely lower** SpO_2 (cold extremities, poor perfusion, nail polish, dark pigmentation) or falsely **reassure** in CO poisoning (SpO_2 reads normal because CO-Hb absorbs at same wavelength).
- **Manual BP:** Trap says inflate cuff "as high as possible" or uses a cuff that's too small (falsely **high** reading) or too large (falsely **low**). Also watch for "deflate rapidly" — correct rate is 2–3 mmHg/sec.
- **Respirations:** Answer that says "tell the patient you are counting respirations" is wrong — patient awareness changes respiratory rate. Count while appearing to assess the radial pulse.
- **Transfer safety:** Trap omits locking wheelchair brakes or uses no transfer belt when needed. **Always** lock brakes and ensure a stable base before moving a patient.
- **Documentation:** Wrong answers skip recording a vital sign or use "WNL" without numeric values. **Record actual measurements** — never leave a VS undocumented.